
STRATEGY & GROWTH PLAN / THE FULL ENGAGEMENT

From clinic to institution

The plan to reach N45-50M a month at an 18-20% EBITDA floor

Every change and every workstream Consult for Africa will deliver at Haven, tied to one growth mandate. Where the business is now, the target, and exactly how the target is reached.

For: Mr Kabir Aregbesola, Mrs Abisodun Alli, Dr Shakirah Saliu, Dr Odedina, Mr Ogochukwu Odum

From: Dr Debo Odulana, Founding Partner, Consult for Africa

Status: Board strategy deck / figures are an illustrative target architecture with stated assumptions



One number governs this plan

Every workstream in this deck is judged against a single growth mandate. If a move does not move the mandate, it waits.

N45-50M

MONTHLY REVENUE

Roughly N540-600M a year, about 2.5x the N15-20M Haven averages today

18-20%

EBITDA FLOOR

A minimum, not a hope: the business must be durably profitable

6 months

TO THE MANDATE

Reached by February, driven by the NICU into the paediatric high season

The mandate is deliberately a floor on profitability, not just a revenue target. Growing revenue while margins stay thin is the trap Haven is in now. This plan grows both, together.



The thesis in four moves

- **Culture is the multiplier.** Haven already has the expensive thing, a staff that cares, confirmed by its own survey. We build the systems that let good people be good without heroics.
- **Capture before you drive.** You cannot bank revenue you cannot record. Billing, tariff and working capital come first, then the growth engines are switched on.
- **Clinic to club.** The membership turns one-off visits into a recurring relationship, and Haven from a place families visit when sick into a paediatric club they belong to.
- **Own the moat, rent the rest.** Own the clinical care, the culture and the differentiated services. Outsource inventory, the lab and the back office to people who do them for a living.

Ten essential growth levers across five jobs, sequenced so the early moves fund the later ones. The trapped N7M and early margin gains pay for the growth.



Where we are, the target, and how we get there

1

WHERE WE ARE

A young hospital with a real culture and immature systems. N15-20M a month, thin margins, and N7M of cash locked up.

2

THE TARGET

N45-50M a month at an 18-20% EBITDA floor, reached in six months, by February, with a clear P&L behind it.

3

HOW WE GET THERE

Ten essential growth levers in five jobs: fill the beds, lift value per patient, free the cash, build the team, and make Haven known.



Haven today: a strong start on young systems

- **A genuine achievement.** Around 18 months in, Haven funds its own salaries, holds a steady patient base, and has earned a real reputation for paediatric care in Ikeja.
- **The expensive thing is already built.** The hardest asset in any hospital, a team that genuinely cares, is in place, as the staff survey confirms.
- **The systems beneath it are immature.** Revenue is captured on what is effectively a nursing handover sheet, the management numbers do not reconcile, and around N7M of cash is locked in receivables and stock.
- **Growth is now outpacing the systems.** As Haven scales into more complex neonatal care, the governance, standards and financial discipline have to be deliberate, not assumed.

This is the good kind of problem: the hard part is done, and what remains is the more tractable work of building systems, best done now while the facility is still young.



The staff survey: culture is a real asset

An anonymous survey of 23 staff, built with reverse-worded items so people could not simply agree with everything. It still came back strong.

16 / 17

RATED SAFETY VERY GOOD OR EXCELLENT
on the first wave of responses

4.6 / 5

WOULD RECOMMEND HAVEN FOR CARE
the highest-scoring item in the survey

Low

FEAR, UNSAFE PACE, RISKY HANDOVERS
the reverse items scored low, the good result

The soft spots were not about safety. They were pay clarity, reward fairness, recent resuscitation training, and staff welfare, health insurance and pension. Staff are telling us where the next investment belongs, calmly, which is the best time to hear it.



The audit: where the systems leak

- **Revenue capture.** The revenue record is effectively a nursing handover sheet, so services delivered are not reliably billed. That gap is pure lost revenue.
- **Working capital.** Around N4.2M sits in HMO receivables and N2.77M in pharmacy stock: roughly N7M of cash locked up while leadership feels cash is tight.
- **Reporting integrity.** Visit counts do not reconcile, the receivables table does not foot, and a 201% pharmacy 'profit' is a markup, not a margin. The board cannot yet trust its own numbers.
- **Licensing & governance.** The establishment licence must be squared with the service Haven now delivers before it scales further into complex neonatal care.

None of this is a failure of effort. It is the absence of a senior operator and the systems one installs. Every one of these is fixable, and fixing them is where the plan starts.



Current economics, honestly stated

N15-20M

REVENUE PER MONTH

What Haven averages today, on a reporting layer that does not yet reconcile

~8%

EBITDA (ESTIMATED)

Thin. 'Barely anything left' after salaries

N7M

WORKING CAPITAL LOCKED

In receivables and pharmacy stock, not in profit

We state the baseline as an estimate on purpose. The first build in this plan, a real billing and reporting system, is what turns these estimates into numbers the board can stand behind. You cannot manage what you cannot measure.

Baseline firmed up by Move C1 (billing & reporting) in the first two months.



The distance to the mandate

TODAY

N18M

8% EBITDA

Thin margin, cash locked

>

~2.6x

THE MANDATE

N46.5M

19% EBITDA

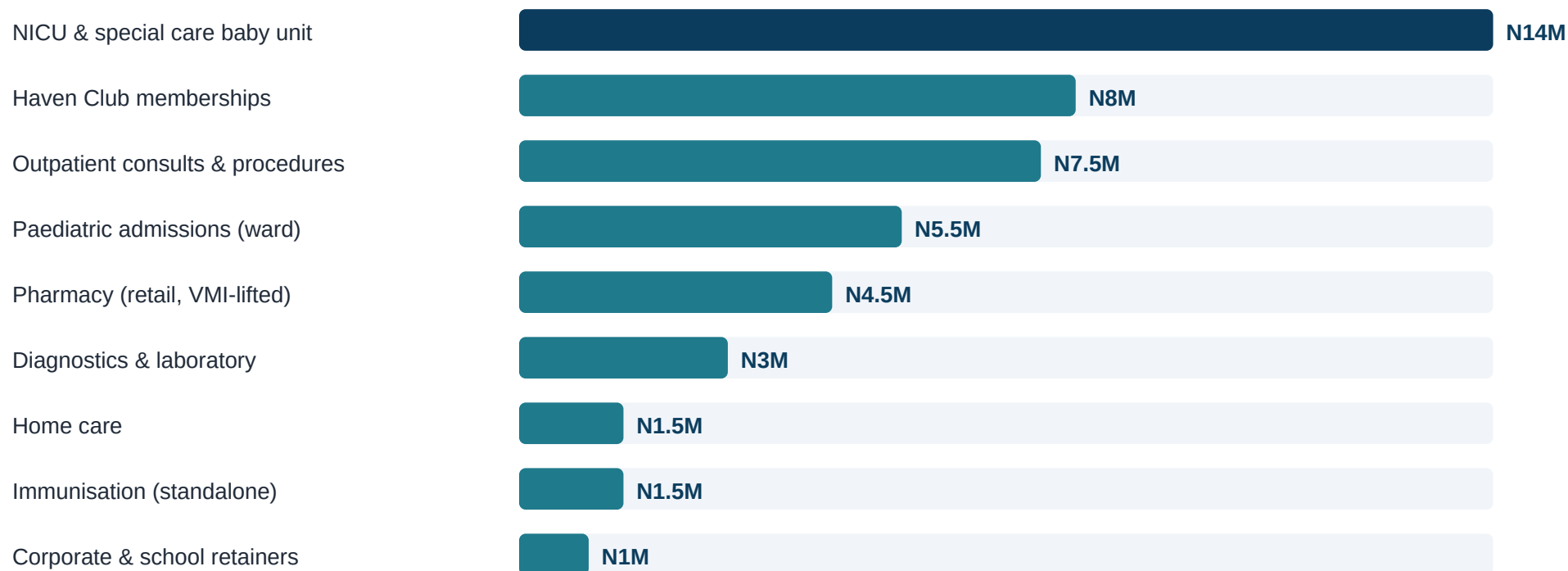
Durably profitable, at scale

About two and a half times, in six months. It is not one heroic leap; it is led by the NICU, which is underused today, driven into the paediatric high season, with the fast fixes underneath funding it.



What the business looks like at target

At steady state, roughly month 24, Haven runs nine revenue lines instead of one, with the highest-margin lines carrying the most weight.

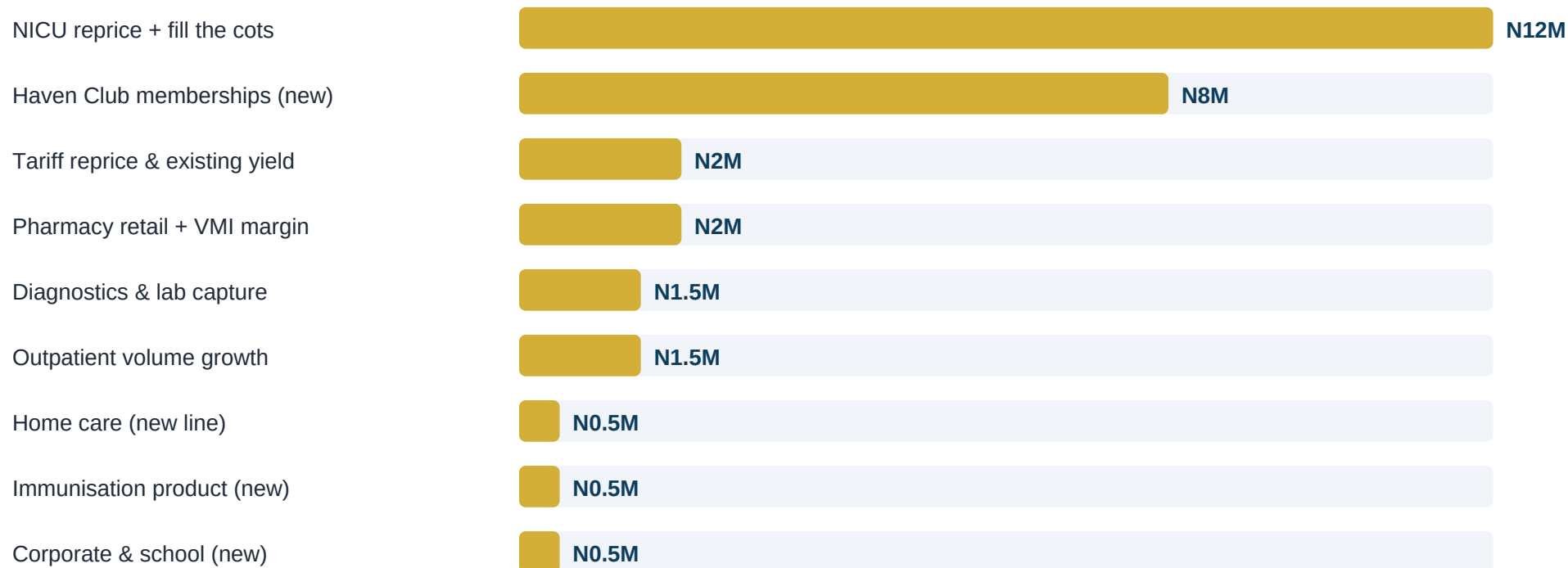


Total ~N46.5M / month (~N558M / year)



How we close the gap

The bridge from N18M to N46.5M a month, ranked by contribution. NICU and the Haven Club do the heaviest lifting.



N18M today + N28.5M of new monthly revenue = ~N46.5M / month



The target P&L, monthly

Revenue

NICU & special care baby unit	N14M
Haven Club memberships	N8M
Outpatient consults & procedures	N7.5M
Paediatric admissions (ward)	N5.5M
Pharmacy (retail, VMI-lifted)	N4.5M
Diagnostics & laboratory	N3M
Home care	N1.5M
Immunisation (standalone)	N1.5M
Corporate & school retainers	N1M
Total revenue	N46.5M

Operating costs

Clinical & nursing salaries	N14.5M
Drugs & consumables (post-VMI)	N7.5M
Other staff: admin, ops, CX, club	N4M
Rent, facility, utilities, power	N4.5M
Outsourced lab & services	N2M
Membership programme & events	N1.8M
Marketing & business development	N1.5M
C4A management fee	N1.9M
Total costs	N37.7M

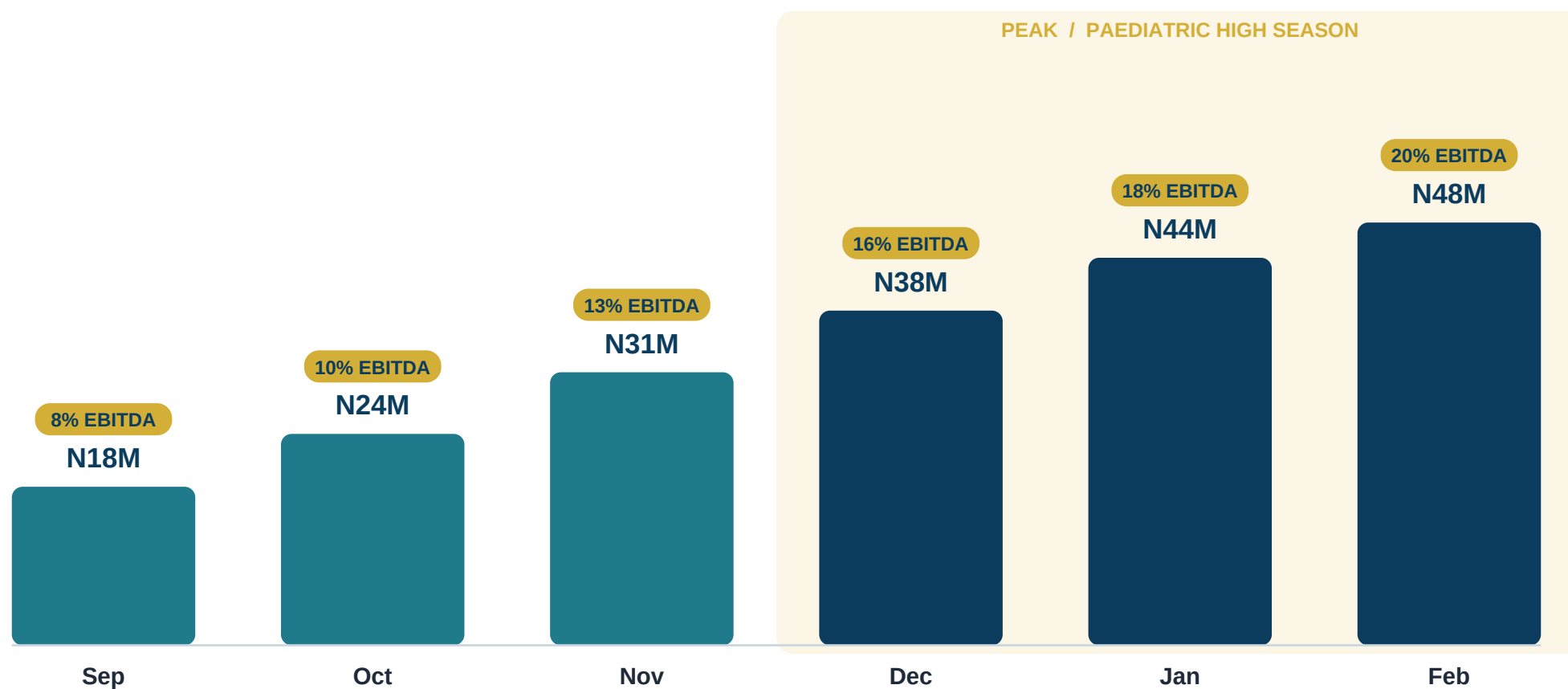
EBITDA

~N8.8M / month = ~19% (above the 18% floor)



Six months to the mandate

Run the fast fixes now and drive the NICU into the paediatric high season, and Haven reaches N45-50M a month at an 18-20% EBITDA floor by February, month six.





Why six months is achievable

From N15-20M today the mandate is about two and a half times, and it is reachable fast for one reason: it is led by the NICU, a price-and-throughput decision, not a slow ramp.

01 NICU is a decision, not a ramp

The price is reset at once and three underused cots fill from a referral drive. High unit value, so it moves the number fast.

02 High margin, so EBITDA follows

NICU is the highest-margin work Haven does, so revenue and the 18-20% floor climb together, not one long after the other.

03 The season is the tailwind

November to February is the paediatric high season. Launch now and demand arrives exactly when the beds and the pipeline are ready.

The fast fixes underneath, revenue capture, the tariff reprice, VMI and receivables, land in weeks, not quarters. Nothing on the six-month path waits for the slow structural build.



The one dependency, and the honest read

The make-or-break: fill the NICU

- **The cots are underused today.** So the peak rests on filling them, the number-one priority from week one.
- **But it is controllable.** Three feeders do it: Ob/Gyn tours and an antenatal pathway, a neonatal transport ambulance, and a referral network, so Haven actively generates referrals, not just waits on them.
- **The season helps.** November to February is the paediatric high season, so the demand tailwind is real if the feeders are built to catch it.

Reach it in six. Then hold it.

Six months reaches N45-50M, at the February peak, on the NICU and the season together. From N15-20M today that is the mandate, hit inside the year.

The job after February is to hold it through the quieter months, converting the seasonal NICU peak into diversified, year-round revenue as the club and the other lines mature. Reaching it and holding it are two tasks, and honesty about that protects the win.

If the referral drive lags, February is ~N38-42M:

still a major step from N15-20M, and the honest downside to plan for.



Five jobs, ten levers

The whole strategy on one slide: ten essential growth levers, grouped into the five jobs that take Haven to the mandate, led by the NICU.

A. Fill the beds

DEMAND

1 Referral portal & analyst 2 NICU growth plan 4 HMO & corporates

B. Lift value per patient

YIELD

6 Reprice the tariff 3 Service expansion (surgery) 5 Haven Club 9 Add-on services

C. Free cash & margin

MARGIN

7 Working capital: vendor-managed inventory and lab outsourcing

D. Build the engine

PEOPLE

8 Cultural transformation and the staff app

E. Make Haven known

AWARENESS

10 Marketing and promotion

Each lever is detailed in the slides that follow.

The revenue model

Nine lines instead of one. A membership club, a repriced tariff, the NICU engine, a retail pharmacy, captured diagnostics, and three new service lines.





From clinic to club

- **Parents buy care because they must.** They renew a club because it removes fear, gives them a named doctor who knows their child, and lets them belong.
- **The want, not the need, is what recurs.** Treating illness is the need. Reassurance, development, community and status are the wants, and the wants are what a parent pays to keep.
- **It is genuine white space.** No Lagos paediatric hospital runs a membership club. Haven can define the category in its catchment.
- **Registration stays and rises.** The one-off registration goes from N10k to N15k for everyone; the club is an optional annual membership on top. Non-members still pay per visit.

The club sells the relationship, not a discount. It also turns Haven from a place families visit when sick into a paediatric health-and-development club they belong to for a decade.



Three plans, by the child's stage

Each plan bundles the essentials, needs and wants of one age, wrapped in the club: a named paediatrician, a 24/7 line, no queue, events, community and milestone moments.

First Year (0-15 mo)

N600k / N1.2M

Full vaccine schedule, well-baby visits, unlimited waived consults, newborn home check, lactation, sleep and weaning support

Growing Child (1-5 yr)

N350-500k

Unlimited waived sick consults, development monitoring, boosters, play sessions, parenting workshops

School Age (5-16 yr)

N180-280k

Annual check with school and sports forms, waived consults, emotional-wellbeing check, activity access

Included: consults, well-child visits, the 24/7 line and events. Billed at a member discount: labs, drugs, admissions, procedures, NICU.



How many we sell, and what it earns

The target is 175 active memberships by month 24, built from the patient base and every new birth Haven touches. Priced to the family wallet, not to an HMO.

Plan	Members	Annual fee	Annual value
First Year (0-15 months)	80	N600k-1.2M	N64M
Growing Child (1-5 years)	55	N350-500k	N23.4M
School Age (5-16 years)	40	N180-280k	N8.8M
Total	175		N96.2M / yr

**That is about N8M a month in membership fees alone, before the labs, drugs and admissions those loyal families also buy.
The ramp: roughly 70 members by month 12, 175 by month 24.**



Reprice the tariff: switch on revenue already earned

This is not a market study. It is capturing income Haven already produces but does not charge for.

- **80.9% of the tariff is priced at zero.** Of Haven's own 2,413-line tariff, four lines in five carry no price, so the service is delivered and never billed. Repricing those lines is immediate, found revenue, not new work.
- **And the priced lines sit 34% below competitors.** Close that gap by raising prices up to 45% overall, in two tranches across the year, against a real peer tariff, so the rise is gradual, defensible, and easy to communicate.
- **Anchor on self-pay, manage the payer mix.** Price on self-pay first; treat HMOs as a separate, much lower schedule. An HMO pays a fraction of cost per child and never covers a NICU stay, so the mix is managed on purpose, not by drift.

The zero-priced lines plus the 34% gap to market make this the fastest, lowest-risk revenue in the plan: about N2M a month to start, and more as the second tranche lands.



NICU: the growth engine

- **The highest-yield asset in the building.** Three cots at about N3M per admission, anchored by Dr Odedina, a consultant neonatologist. It is the highest-margin work Haven does, and today the cots are underused.
- **Getting the basics right first.** Scaling neonatal care rewards putting the governance, standards and team readiness in place first, so the unit runs full with confidence. That is the people and quality work in Workstream B.
- **Then fill it, deliberately.** Because the cots are underused today, filling them is the single highest priority. Three practical, controllable feeders do it, next slide.

Repriced and filled, NICU is the single biggest lever in the plan: about N14M a month at target, and the engine of the six-month peak.



Fill the NICU: three practical feeders

Haven does not take deliveries; obstetricians keep their delivery and antenatal revenue. The play is to win the baby when it needs the NICU. Three controllable feeders do it.

1 Ob/Gyn tours + antenatal pathway

Welcome obstetricians to tour the NICU with Dr Odedina and build confidence in the capacity. Then co-plan the high-risk pregnancies antenatally, so babies likely to need neonatal care are identified early and pre-booked. They keep the delivery; Haven gets the baby that needs more.

2 A neonatal transport ambulance

A NICU-equipped ambulance that goes out and collects sick and preterm babies from other facilities. A mobile NICU is a feeder, a paid transport service, and a trust-builder with referring hospitals at once.

3 A referral network

Structured relationships with maternity units that refer complications in, backed by visible governance so they trust Haven with a newborn who needs more.

The digital marketing agency already in place drives demand into all three, held to booked leads and admissions, not just posts and reach. Fill the cots and the six-month peak is real.



Pharmacy and diagnostics: margin, not cost centres

Pharmacy as a retail line

- **Vendor-managed inventory.** A consignment model frees the N2.77M tied up in stock and ends stockouts: the vendor owns the stock and carries the expiry risk.
- **Run it for margin.** Proper attach discipline and formulary management turn the pharmacy from a cash trap into a contributor.

Bridge: ~N3M / month

Diagnostics, capital-light

- **Point-of-care in-house.** Keep the tests that change management within the hour, including a bedside bilirubin analyser for jaundice.
- **Send the rest out.** An accredited reference partner runs the specialised tests; Haven captures a transparent markup, no kickbacks. Capex becomes opex.

Bridge: ~N2.5M / month



New and growing service lines

New revenue that also deepens the club and the family relationship.

Home care

Newborn home checks, post-discharge follow-up, home sample collection, and home phototherapy for jaundice, which no one in Lagos offers.

~N1.5M / mo

Immunisation product

Vaccination marketed as a named, reliable product line, the reason young families choose and keep a paediatric relationship.

~N1.5M / mo

Corporate & school

Recurring paediatric retainers with schools and employers, starting from the Toddler Town relationship.

~N1M / mo

Alongside growing the allergy & asthma and sickle-cell clinics already running, and adding therapy, dentistry and imaging over time.

WORKSTREAM B

People & culture

The multiplier. Protect the culture Haven already has, and build the systems, benefits and incentives that make good care self-sustaining.





Culture: the multiplier

- **Start from strength.** The survey shows the culture is sound, so the work is to protect and deepen it, not rebuild it.
- **Install a just-culture standard.** So a reported error fixes the system rather than blames the person, the one culture item the survey flagged as softening.
- **Make good work automatic.** Shift-level clinical routines and standards of work so safety and efficiency do not depend on who is on shift.

Culture is the only lever that makes every other lever cheaper. Underfund it and everything above it, the club, the NICU, the growth, gets harder.



A staff app and a welfare structure

The survey's clearest message was welfare. We answer it in a structured, affordable way, because at this stage the team stops being a cost and becomes the engine.

The staff app

- **The whole HR function, in one app.** Onboarding, rota and shift swaps, leave, payslips, appraisals and KPIs, training and compliance, incident reporting and benefits, so HR runs on a system, not on paper and memory.
- **Recognition and standards, visible.** The standards of work, peer recognition and rewards live where the team already is, reinforcing the culture every day.

The welfare structure

- **What staff asked for.** Health insurance, a pension, fair and timely pay, and clinical training, phased so it is affordable as revenue grows.
- **Retention is the return.** Losing a good clinical lead is months of ward instability. Welfare is what keeps them.



Redesign the incentives

- **Fix what is already on the board's list.** The staff commission structure and the job descriptions and KPIs are already awaiting approval. They are the incentive and accountability tools a strong culture runs on.
- **Reward quality and ownership.** Not activity or volume alone. The survey showed reward fairness is one of the softest scores, so this is felt, not theoretical.
- **Make ownership real.** Clear responsibility and a sense of ownership for how well Haven runs, which the survey found people want and partly lack.

Incentives aligned to quality and ownership are what make the culture sustain itself after we step back. Without them, every improvement depends on someone chasing it.



Close the clinical-training gap

- **Resuscitation first.** Recent resuscitation training, BLS, PALS and NRP, was the lowest clinical-readiness score in the survey. For a paediatric and neonatal centre that is a live safety gap, closed with a standing training cadence.
- **A capability ladder.** Structured competencies for nursing and clinical staff, tied to the standards of work and to progression.
- **A credential the market trusts.** Training that also underwrites the NICU referral pipeline and the accreditation pathway.

Capability is also a revenue enabler: a team trusted with complex neonatal care is what lets Haven scale the highest-margin line it has.



A senior operations leader: the keystone

- **Almost every move needs an owner who is not a founder.** The audit's plainest finding was the absence of a senior operator. This is the person who runs the billing system, the vendor contracts, the referral relationships and the numbers.
- **Define the role, then recruit into it.** The role and its measures are set first, so the hire lands into clarity rather than chaos.
- **Sourced and vetted.** Consult for Africa can source and vet the person through our workforce platform, including senior operator and diaspora profiles.

Without this hire the whole plan depends on the founders' own bandwidth, which is exactly the constraint the plan exists to remove.

Process & quality

The plumbing the whole business stands on: real billing, freed cash, outsourced non-core, clinical governance, and a patient experience worth paying for.





Real revenue capture and reporting

- **Replace the handover sheet.** A proper capture and billing point so every service delivered is recorded, priced and billed. This is the foundation the entire revenue side stands on.
- **The first build.** The membership, the repriced tariff, the pharmacy attach and the NICU repricing all assume what Haven does is what Haven bills. Today it is not, and that gap is pure lost revenue.
- **Numbers the board can trust.** A management reporting layer so the figures reconcile and the board can see the business at a glance.

Nothing else in the revenue plan pays out reliably until this is fixed. It is Move number one for exactly that reason.



Free the N7M that is locked up

N4.2M

IN HMO RECEIVABLES

Recovered, then kept moving by a monthly discipline

N2.77M

IN PHARMACY STOCK

Freed by moving to vendor-managed, pay-on-consumption stock

Week 1

RECOVERY BEGINS

A push on the two largest HMO balances as a quick win

This is the fastest money in the plan. The cash is not missing, it is on the shelf and in the HMO ledgers, and recovering it helps fund the growth without a large new injection.



Own the core, outsource the rest

- **Keep what is truly Haven's.** Clinical care, the culture and the standards stay in-house. They are the brand.
- **Rent what is not.** Inventory and pharmacy supply to a vendor-managed partner, the lab to an accredited reference partner, and payroll, biomedical maintenance, security and laundry to service contracts.
- **Buy focus and cash.** Outsourcing converts fixed headaches and trapped capital into predictable service costs, and frees the founders' time for care, culture and growth.

Every hour a founder spends supervising a generator or a laundry is an hour not spent on the things only Haven can do.



Quality, governance and the licence

- **Resolve the licence first.** Square the establishment licence with the service Haven delivers. It gates everything above the stabilise stage.
- **Install clinical governance.** Formal standards, incident-and-learning systems, and the accountability that a growing facility taking on complex care must have.
- **Earn accreditation.** An external accreditation an HMO or corporate client recognises, which unlocks pricing power, referrals and trust.
- **Mature the board.** Separate ownership, oversight and management, the cleanest way to handle an adviser who also sits on the board.

Governance feels like overhead until the moment it is the only thing standing between Haven and a serious event. Then it is the whole business.



A patient experience worth paying for

- **Design the journey.** From booking to arrival, care, discharge and follow-up, mapped and smoothed, because the experience is what drives repeat visits and referral.
- **No queue, unhurried, communicated.** Same-day access, appointment discipline, and proactive communication, exactly what affluent parents rank above price.
- **Measure it.** A simple satisfaction and net-promoter pulse so experience is managed with data, not assumed.

Experience is the club's product as much as the medicine is. It is also the cheapest marketing Haven has: a parent who felt seen tells every other parent they know.

WORKSTREAM D

Business development

Make the growth deliberate: a referral engine, a funnel that captures families before birth, a brand that means something, and the partnerships that deliver it.





The growth engine

Referral & Bump-to-Baby

- **A referral pipeline.** Structured relationships with maternity units that feed the NICU, and with clinicians who refer complex cases.
- **Start before birth.** Antenatal and newborn-prep classes and a postpartum circle capture expectant parents, when loyalty is set, and channel them into the First Year plan.

Brand & partnerships

- **Direct the agency you already have.** The digital marketing agency in place is pointed at the club, the maternity offer and NICU awareness, and held to booked leads and admissions, not just posts and reach.
- **A brand that means something.** The paediatric and neonatal centre for complex care in Lagos, built on one excellent site before any thought of a second.



The six-month roadmap

Sequenced so the fast, cash-releasing moves in month one fund the NICU drive that peaks the revenue by February.

Month 1

Stabilise & capture

Billing & reporting live, tariff repriced, licence confirmed, receivables recovery, VMI started, NICU repriced, referral drive begun

Months 1-2

The NICU drive

Referral relationships with maternity units, neonatal staffing and governance to run three cots full safely, operations leader in post

Months 2-4

Switch on the lines

Haven Club launch, pharmacy retail and lab outsourcing, staff app and welfare, immunisation, home care, HMO renegotiation

Months 4-6

Peak the season

NICU full into the high season, all lines live, revenue peaks at N45-50M by February at the EBITDA floor



The founders' survey: a strongly aligned board

Four of five founders have responded (Dr Odedina outstanding). The survey forced trade-offs rather than agreement scales, so where they line up, it is real.

- **Reinvest, not distribute.** All four would put the surplus back into Haven rather than take it out now.
- **Hand daily management to professionals.** Unanimous, which is the mandate for the senior operations leader this plan hires.
- **Perfect one site, one flagship.** All four want to perfect one site before growing, and all four want one exceptional hospital, not a multi-site group.
- **Invest in staff as the engine.** All four chose staff investment over protecting margin.
- **Fund revenue and growth first.** The N100 split puts revenue systems and growth ahead, with quality and licensing agreed as the lower near-term priority.

On the fundamentals the board is strikingly aligned: reinvest, professionalise, perfect one site, back the staff. That shared foundation is what makes committing to a plan possible.



...and the real conversations still to have

- **How high to climb.** Two founders aim to stabilise, two aim for a centre of excellence, and they misread each other on it. The ambition itself is not yet settled.
- **The senior operations leader.** Everyone wants professional management, but how much to fund it now splits widest, from 5 to 30 of every 100. One founder names hiring a GM as the single most important move.
- **Build in-house or outsource.** An even split, two each way.
- **Pace.** Three of four lean to protecting harmony over pushing hard, worth weighing against any pressure to hit the number fast.

The hidden gap: all four chose to invest in staff, yet half assume their fellow founders would protect margin instead, and staff is funded below revenue and growth. That is the trade-off one founder named as the one being quietly avoided. It is the board's to resolve, not ours, and best resolved out loud.



How C4A delivers, and how we measure it

- **Yours to own, ours to enable.** Consult for Africa sets the targets, the levers and the expectations, and runs the delivery day to day. The strategy and the decisions stay the founders'. We hold the standard and the pace; we do not take over ownership of your business.
- **One dashboard, honest numbers.** Once billing is live, the board sees a single monthly report: revenue by line, EBITDA against the floor, cot occupancy, membership count, receivables days, and the experience pulse.
- **Reviewed against the six-month ramp.** Every month is judged against the plan: revenue and EBITDA versus target, and each feeder and move tracked, so course corrections happen early.

Transparency is part of the standard we are setting for the facility: the pricing, the discount and the adviser's board seat are all disclosed in full to every owner.



The risks, and how we hold them

Execution capacity

The senior operations leader and clear sequencing stop the plan resting on the founders' bandwidth.

Membership take-up

Priced to the family wallet and tested with real parents before launch; value published so it is a visible win, not a gamble.

Club over-build

Kept asset-light. A heavy family club collapsed elsewhere on capped membership versus fit-out; Haven leans on its clinical core and partners.

Adverse selection

Enrolment windows, waiting periods and a well-child framing stop the club attracting only the already-sick.

Baseline uncertainty

The current numbers are estimates until billing is live; the plan makes that its first build rather than papering over it.

THE MANDATE, RESTATED

N45-50M a month. 18-20% EBITDA. Reached in six months, into the season.

From N15-20M today it is about two and a half times, led by the NICU into the paediatric high season, with the fast fixes funding it. The targets and the levers are ours to set and deliver; the strategy and the calls stay yours. The first moves start now: the tariff and billing, the NICU feeders with Dr Odedina, and the operations hire.

Consult for Africa

Healthcare transformation partner

hello@consultforafrica.com / +234 913 813 8553 / consultforafrica.com

Lagos and Abuja, Nigeria